

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°144 /MVE-BDBV/05/10/2026



Report date: October 5, 2026



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**HIGHLIGHTS** (last 24 hours)

Sixty-two (62) new confirmed cases of Ebola Virus Disease (EVD), including 20 deaths. Community transmission cases were reported. These cases originated from the provinces of Ituri (30 cases), North Kivu (26 cases), Haut-Uélé (5 cases), and Tshopo (1 case). In addition, 10 deaths were recorded in Ebola Treatment Centers (ETCs).

• A total of 311 new samples were analyzed; 62 of these samples tested positive for the virus Bundibugyo, or 19.9% positivity.

ACCUMULATIVE CAS 8 665	ACCUMULATIVE DEATH CONFIRMED 4 178	LETHALITY 48,2 %	PATIENTS IN ISOLATION/CTE 890	ACCUMULATIVE HEALED 2 259	TRACKING RATES CONTACTS 80,4% (Of the Day)
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Affected provinces and affected health zones

**7 Provinces affected**

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



**64 out of 167
health zones affected**
(38,3%)

Summary of key points from the last 24 hours

In the past 24 hours, **62 new confirmed cases** of Ebola Virus Disease (EVD) including **20 community deaths** have been reported in 4 provinces: Ituri (30 cases), North Kivu (26 cases), Haut-Uélé (5 cases) and Tshopo (1 case).

In addition, **10 deaths were recorded in the CTEs** (6 in Ituri, 3 in North Kivu) and in Haut Uélé (1), which brings the total number of confirmed deaths for the period under consideration to 30.

In contrast, **24 patients (17 in Ituri, 3 in Haut-Uélé, 2 in Bas-Uélé, 1 in North Kivu and 1 in South Ubangi)** were declared cured after obtaining two negative results in the Ebola control test.

As of October 5, 2026, the DRC has recorded **8,665** confirmed cases and **4,178** deaths, representing a case fatality rate of **48.2%**. Ituri remains the epicenter of the epidemic with **74.8%** of cumulative confirmed cases and **48.4%** of new confirmed cases reported in the last 24 hours.

64 out of 167 health zones (38.3%), spread across **seven provinces**, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 5, 2026.					
Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	30	6480	2 989	46,1%	28/36 (77,8 %)
North Kivu	26	1755	1 013	57,7%	17/34 (50,0 %)
Haut-Uélé	5	364	153	42,0%	7/13 (53,8 %)
Boss	1	51	17	33,3%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	10	4	40,0%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	62	8 665	4 178	48,2%	64/167 (38,3 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 5, 2026							
Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cases (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6480	2989	46,1%	30	15	6	21
Give	11	1	9,1%				0
Underwood	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	210	46	21,9%	2			0
have	2	2	100,0%				0
Bunia	1783	588	33,0%	4	2	2	4
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	83	31	37,3%				0
Gethy	9	5	55,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	175	107	61,1%				0
Liter	294	147	50,0%				0
Logo	14	5	35,7%	1	0	0	0
Fight	34	9	26,5%	0	0	0	0
distributed	4	1	25,0%				0
Washing	37	16	43,2%				0
Darkness	195	97	49,7%	19	13		13
Mangala	365	157	43,0%			0	0
Mongbwali	707	336	47,5%			1	1
Nanny	311	175	56,3%				0
Nizi	788	282	35,8%	2			0
Legumes	130	37	28,5%				0
Jungle	14	5	35,7%	1			0
Rwampara	1115	383	34,3%	1		3	3

Tchomia	76	25	32.9%					0
A valves	THAT	493	THAT					0
North Kivu	1755	1013	57,7%	26	5	3		8
Me	400	264	66,0%	7	3	1		4
Farm	20	7	35,0%					0
Butembo	283	180	63,6%	3	0			0
Ten	1	0	0,0%					0
Kalunguta	61	21	34,4%	1	0			0
Cut	606	372	61,4%	6	1	1		2
Source	3	2	66,7%					0
Kyondo	36	20	55,6%					0
Lubero	22	11	50,0%					0
Be anxious.	51	20	39,2%	5	1	1		2
Alimbong	4	2	50,0%					0
Manguredjipa	13	5	38,5%					0
Masereka	32	11	34,4%					0
The museums	138	69	50,0%					0
Mutwanga	17	3	17,6%	1	0			0
Oicha	33	16	48,5%	2	0			0
Vuh	35	10	28,6%	1	0			0
Haut-Uélé	364	153	42,0%	5	0	1		1
Government								
Mangbetu	43	19	44,2%	0	0	1		1
Dumb	1	1	100,0%					0
Gombari	3	1	33,3%					0
Mathematics	102	44	43,1%					0
Lining	92	43	46,7%	2	0			0
Beans	4	2	50,0%					0
Ordinary	119	43	36,1%	3	0			0
Boss	51	17	33,3%	1	0	0		0
Bafwasende	7	1	14,3%					0
Kabondo	5	1	20,0%					0
Lubunga	1	0	0,0%					0
Maquis-Kisangani	27	11	40,7%					0
Mangobo	8	3	37,5%					0
Boss	2	0	0,0%	1	0	0		0
Wanie-Arugula	1	1	100,0%					0
South Kivu	3	1	33,3%	0	0	0		0
Miti-Murhesa	3	1	33,3%					0
Bas Uélé	10	4	40,0%	0	0	0		0
Blind	1	1	100,0%					0
Viadana	6	1	16,7%					0
Walk	3	2	66,7%					0
Sud Ubangi	2	1	50,0%	0	0	0		0
Hair	2	1	50,0%					0
Total	8665	4178	48,2%	62	20	10		30

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for October 5, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	64	3	67	12	3	52	0	15	3
Bas Uélé	187	3	190	29	3	158	0	32	0
Ituri	1 432	76	1508 164		75	1 268	1	239	132
North Kivu	1 083	36	1119 168		32	915	4	200	133
Sud Ubangi	31	0	31	0	0	31	0	0	0
Boss	178	0	178	12	0	166	0	12	0
Total General	2 975 118 3	093 385			113 2 590		5	498	268

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

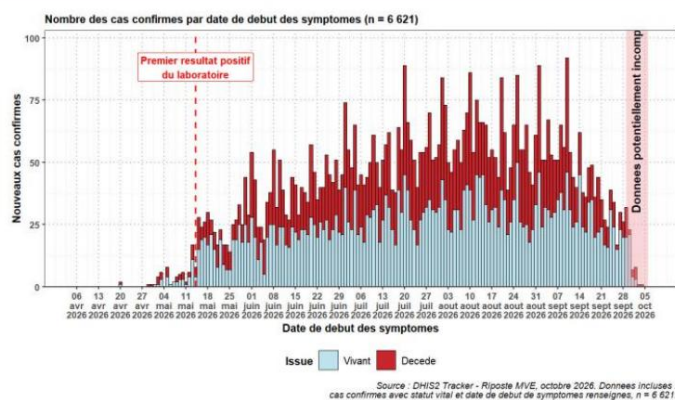
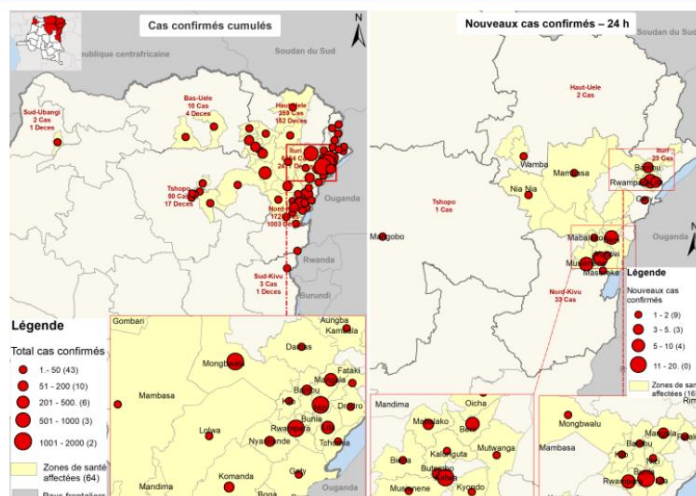


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

For the day of October 5, 2026, **3,093 alerts (100%)** were verified, of which **498 (16.1%)**

2,595 (83.9%) were validated as suspected cases and invalidated. All 498 validated suspected cases were investigated (100%). Of these, **268** were transferred to CT/CTE

Of the **29,535 contacts currently being monitored**, **23,741** were seen in the last 24 hours, representing a follow-up rate of **80.4 %**, up from the previous day (**79.7 %**), but still below the required threshold of **95.0 %**. This rate was **83.4% (10,703/12,830)** in Ituri, **76.8% (10,584/13,790)** in North Kivu, **85.6% (1,616/1,888)** in Haut-Uélé, **80.2% (679/847)** in Tshopo, **86.8% (118/136)** in Bas-Uélé and **93.2% (41/44)** in Sud-Ubangi.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Two (2) alerts were notified to the PoE/PoC during the day of October 5th, one (1) in Ituri and one (1) in North Kivu. In Ituri, the body of a female deceased, traveling from Uganda to Tchomia for burial, was intercepted at the Tchomia PoC (Kikosa Health Area, Tchomia Health Zone), then validated and swabbed, with the result pending (body swabbed/secured: 100.0%; 1/1). In North Kivu, the alert notified to the PK5 PoC concerned a 33-year-old Ugandan driver, traveling from Beni to Uganda, presenting with vomiting, diarrhea, chills, and asthenia; validated as a suspected case, he could not be evacuated due to the lack of an ambulance (transfer rate: 0.0%; 0/1).

Table 4. Status of key PoE/PoC indicators by province, as of October 5, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	96,7% (58/60)	95,2% (20/21) 44,4% (8/18)		86,9% (86/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	156 717	99 704	9 107	265 528
% of travelers screened	96,0%	97,6%	97,4%	96,6%
% of travelers who washed their hands	91,1%	97,6%	97,7%	93,7%
% of travelers made aware	99,2%	97,6%	100,0%	98,6%
Number of alerts notified	1	1	0	2
Number of validated live alerts (suspected cases)	0	1	0	1
Number of bodies recovered today	1	0	0	1
% of suspected cases transferred to CTE/CT	THAT	0,0%	THAT	0,0%
% of lifeless bodies swabbed/secured	100,0%	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	THAT	100,0%	THAT	100,0%

2.2.2. Challenges

In Tshopo, the completeness of reports remains low (44.4%; 8/18), with ten private ports in Makiso-Kisangani not operating on Sundays due to high taxes, and 2.6% of travelers refused screening and 2.3% refused handwashing. In Ituri, 4.0% of travelers were not screened, and the proportion of travelers who washed their hands fell to 91.1% from 97.2% the previous day. In Haut-Uélé, 159 travelers (1.0%) refused screening and 175 (1.1%) refused handwashing, mainly at the Points of Control (PoCs) of PK10 Route Poko, Magambe, and Boh, amid rumors that the water and infrared thermometers contained the virus. In Bas-Uélé, only 19 PoCs out of 41 notification sites are functional (46.3%), no PoE is reinforced (0/11) and the PNHF services are not operational.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 30 new positive results** (15 living and 15 deceased) out of 146 samples received and analyzed (**positivity: 20.5%**).
- **North Kivu: 26 new positive results** (21 living and 5 dead) out of 116 samples received and analyzed (**positivity: 22.4%**).
- **Haut-Uélé: 5 new positive results** (5 live) out of 25 samples received and analyzed (**positivity: 20.0%**).
- **Tshopo: 1 new positive result received** (1 live sample) out of 14 samples received and analyzed (**positivity: 7.1%**).
- **Bas Uélé: 10 samples** (10 blood samples) received and analyzed at the Buta laboratory, **all are negative income**.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 20 out of 40 rings were opened (50.0%), 80 out of 80 households were decontaminated (100%), and 6 out of 6 health facilities were decontaminated (100%); no households, social economy enterprises, or schools received kits. Of 96 blood donation alerts (including 6 postponements), 59 blood donations were carried out (61.5%) and 65 bodies were swabbed; 29 failures and 8 postponements were recorded by the end of the day.
- **North Kivu:** 45 out of 63 rings were opened (71.4%), 18 out of 63 households were decontaminated (28.6%), and 37 out of 61 health centers were decontaminated (60.7%). None of the 18 decontaminated households received a kit, and 35 IPC scorecard assessments yielded an average score of 37.8%. Of 45 EDS alerts and 42 reports, 45 EDS were carried out and 45 bodies were swabbed. Training of 40 PPLs on IPC/WASH/EDS was completed in Oicha with WHO support, and payments for 10 decontamination teams and 73 community supervisors began with UNICEF support.
- **Tshopo :** Two infection control rings were opened in Mangobo, two community health centers (CS Salama and Clinique Kawaki, Mangobo health zone) and the two households of case #48 were decontaminated, with the incineration of mattresses, mosquito nets, and sheets; 14 out of 14 community health centers were monitored in 5 health zones, with 136 service providers briefed (100%). Cumulatively, 39 out of 39 community health centers (100%) and 30 out of 33 households (90.9%) that had hosted a confirmed case were decontaminated within 48 hours. Regarding the blood and casualty incidents (BSIs), two alerts were received (Emergency Treatment Center and Makiso-Kisangani General Referral Hospital), one BSI was carried out, and two bodies were swabbed.
- **Bas-Uélé :** 13 out of 13 ESS (100%) were monitored in Viadana (2), Buta (3) and Ganga (8), 29 out of 30 providers (96.7%) and 25 out of 25 hygienists (100%) were briefed; the training of 4 out of 4 EDS teams was completed in Buta with the support of the Red Cross, of which 3 are already activated, and 3 swabs were carried out in Dakwa, Koteli and Viadana.

2.4.2. Challenges

In **Tshopo**, five bodies could not be swabbed, infection prevention and control (IPC) supplies and trained personnel remain insufficient, and the incinerator at the Makiso-Kisangani General Referral Hospital is still out of service. In **Bas-Uélé**, the reporting of community deaths remains low and delayed, and community health workers lack waste management areas, hygienic latrines, garbage cans, and handwashing facilities.

2.5. Holistic care

2.5.1. Main actions

- In **Ituri**, 86 new admissions were recorded compared to 92 discharges, including 17 recoveries, 65 non-cases, and 10 deaths (6 of which were confirmed). A total of 366 patients remained in isolation at the end of the day. The bed occupancy rate was 36.1%; 312 out of 345 patients (90.4%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (13/13), in daycare (6/6), and in the community (46/46); 37 patients are in critical condition.
- In **North Kivu**, 155 new admissions (21 confirmed and 134 suspected) were recorded against 85 discharges, including 1 recovery in Katwa, 78 non-cases, and 6 deaths (3 confirmed and 3 suspected). In total, 417 patients (139 confirmed and 278 suspected) remained hospitalized, including 271 in standardized facilities. The bed occupancy rate was **74.2%** (271/365); 118 patients received hot meals, 16 separated infants were on breast milk substitute (100%), and 3 patients were in critical condition (Beni and Kitatumba Ebola Treatment Centers); 10 out of 14 ambulances were used, and all 33 requested trips were completed.
- In **Tshopo**, 3 new admissions (1 confirmed and 2 suspected) were recorded against 2 discharges of non-cases. 18 patients (8 confirmed and 10 suspected) remained in isolation at the end of the day, representing an occupancy rate of 50.0% (18/36 beds); the 18 patients and 47 people with disabilities received hot meals and 260 people have received post-exposure prophylaxis since the start of the response.
- In **Bas-Uélé**, 29 new admissions of suspected cases were recorded in the non-standard isolation sites of Ganga-Dingila (12), Bondo (7), Poko (5), Buta (3) and Limba (2), compared to 2 discharges of recovered patients in Limba; 29 suspected cases remained in isolation at the end of the day.
- In **Haut-Uélé**, 9 new admissions (3 confirmed and 6 suspected) were recorded, compared to 8 discharges (7 non-cases and 1 confirmed death at the Boma Mangbetu Ebola Treatment Center). A total of 60 patients (31 confirmed and 29 suspected) are in isolation in Ebola Treatment Centers, representing an occupancy rate of 39.2%.

(60/153 beds); 3 recovered patients were discharged from the CTEs of Isiro (1) and Wamba (2) and 57 patients, 76 accompanying persons and 80 PPLs benefited from hot meals.

2.5.2. Challenges

- **In Ituri**, the low capacity of the CTE/CT in Mambasa, Kilo, Mangala, Mandima, Sota and Lolwa, the presence of 37 critical patients and the lack of food support for 33 patients in Lolwa (14), Mambasa (9) and Mandima (10) remain a concern.
- **In Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- **In Tshopo**, the lack of delivery kits and beds and ultrasound equipment at the CTE, the insufficient number of beds at the experimental treatment center in Makiso-Kisangani and the demotivation of 12 out of 29 ambulance drivers (41.4%) weaken the care provided.

2.6. Vaccination

2.6.1. Main actions

- **In Bas-Uélé**, the vaccination of PPL and TPL continued in the health zones of Buta, Aketi and Viadana, with the pre-registration of religious leaders, RECO, traditional healers, motorcyclists and hunters, bringing the provincial total to 1,333 people vaccinated out of a target of 13,202 (10.1%), mainly in Buta (620; 21.6%) and in Ganga (273; 20.5%).
- **In Tshopo**, 4 out of 5 ZSs were served with Ervebo vaccine (Makiso-Kisangani: 400 doses, Tshopo: 100, Mangobo: 50 and Kabondo: 50) and vaccination started in Makiso-Kisangani and Mangobo ZSs.

2.6.2. Challenges

In Tshopo, the health zones of Tshopo and Kabondo, although supplied with vaccines, have not yet vaccination has started.

In Bas-Uélé, the UCCs are not operational, the validated micro-plan and vaccination funds are not available, and the vaccination team's vehicle deployed in Viadana needs to be repaired and supplied with fuel.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo,

the evaluation of the Sendwe Health Center (Malkia Health Center, Tshopo Health Zone), from which the 18th confirmed PPL originates, revealed that no staff were vaccinated (0/11), the absence of several registers, the non-use of the partogram, the absence of an incinerator and placenta hole and an ineffective triage area; the recovery plan for the Boyoma Health Center was developed and the follow-up of PPLs at the CTM and CTE as well as that of recovered patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.

- **In Bas-Uélé**, the Limba ESS (Viadana Health Zone) was evaluated on the functionality of its management and community participation bodies, by comparing the revenues and meetings of the CODI and CODESA held from May to September 2025 and 2026

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 23,782 home visits reached 105,316 people; 998 educational talks brought together 20,218 people; 112 mass awareness campaigns reached 6,153 people; and 5 advocacy efforts reached 29 authorities and leaders. A total of 389 community alerts, including 142 deaths, were reported; 308 out of 319 feedback reports (96.6%) were collected; and 5 out of 6 incidents (83.3%) were resolved within 48 hours.
- **In North Kivu**, 10,654 home visits reached 34,721 people, 230 educational talks reached 7,449 people and 63 mass communication sessions reached 8,368 people; 201 out of 750 expected community alerts (26.8%) were notified, 13 out of 17 feedbacks (76.5%) were resolved within 48 hours and 910 out of 4,750 expected people (19.2%) engaged in community actions. • **In Tshopo**, 47,270 people (21,092 men and 26,178 women) were sensitized during 7 mass sessions in Bafwasende, Tshopo, Mangobo and Makiso-Kisangani, 3 educational talks reached 187 people and a community debate, with the testimony of a healed person, brought together 104 people at the Christ-Roi parish in Mangobo; 4 broadcasts were held, 258 posters and 121 leaflets were distributed.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In North Kivu**, the last 30 confirmed cases out of 33 (90.9%), 246 confirmed cases under follow-up, 223 new suspected cases, and 676 suspected cases under follow-up have benefited from SMSPS interventions. Teams assisted 66 households with public health activities, supported 87 people living with HIV, monitored all 8 recoveries in the community (100%), conducted psychoeducation for 304 people (117 men, 146 women, and 41 children) with UNICEF support, and announced 145 results, including 39 positive cases.
- **In Ituri**, 18 out of 23 confirmed cases (78.3%) from the previous day, 111 confirmed cases under observation out of 121 (91.7%), 42 new suspected cases out of 70 (60.0%), and 105 suspected cases under observation out of 134 (78.4%) benefited from SMSPS interventions. The teams supported 338 caregivers and 43 people living with HIV/AIDS (PLHIV), monitored 20 out of 24 recoveries (83.3%) in the community and 15 out of 15 children in daycare, provided support to 14 out of 17 separated and orphaned families (82.4%), reintegrated 11 out of 16 recoveries (68.8%), and announced 39 results, including 18 positive cases.
- **In Tshopo**, the new confirmed case and the 5 confirmed cases under follow-up at the CTE Météo, as well as 6 new suspects out of 6 and 1 suspect under follow-up out of 1, have been taken care of (100%); the teams supported 17 PPLs and 3 households, reintegrated 1 recovered, carried out psychoeducation for 1,325 people, including 1,244 in Bafwasende, lifted 3 out of 3 resistances and announced 3 results, including 1 positive; 5 out of 7 ZSs that notified are covered (71.4%).
- **In Haut-Uélé**, 26 confirmed cases under follow-up, 7 new suspected cases and 22 suspected cases under follow-up benefited from SMSPS interventions in the CTEs of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 67 caregivers, 61 PPLs, 14 children separated in the community and 16 household members, supported 1 EDS and announced 12 results, including 7 positive ones.
- **In Bas-Uélé**, SMSPS activities continued in the Viadana (Limba) health zone: 1 PPL and 9 household and community members were supported, 2 discharged healers are being monitored and 1 result was announced (negative).

2.9.2. Challenges

- **In Ituri**, only 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone has no SMSPS coverage, which requires strengthening the capacities of psychologists and APS/TPS to ensure interventions around cases.
- **In Bas-Uélé**, SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.
- **In Haut-Uélé**, the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa
- **In Tshopo**, there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

2.10. Logistics

2.10.1. Main actions

- **In Bas-Uélé**, deployment of surveillance teams in Ganga and Viadana for contact tracing, alert verification and active search, support for the registration of SGI members, security and strategic coordination, and continuation of the rehabilitation of the meeting room and offices of the DPS as well as the construction of the HGR Buta transit center, the perimeter walls of which are completed.
- **In Tshopo**, development of the logistics monitoring checklist, monitoring of the construction of the Bafwasende CTE and the isolation of Konga-Konga, extension of the Weather CTE (2 rooms completed out of 4 planned) and support for the mobility of the PCI, PEC, Communication and PoE/PoC pillars; the mobility need of the ZS remains estimated at 154 motorcycles.
- **In Ituri**, 17 out of 33 ambulances were mobilized and 21 patients were evacuated to care facilities.

- **In North Kivu**, 5 small houses have been erected on the Musienene CTE construction site, The expansion work at the Katwa CTE has been completed with the installation of doors and windows, and the IPC medicines and supplies provided by UG-PDSS have been delivered to the health zones of Musienene and Masereka.

2.11. Security

2.12. Main actions

- **In North Kivu**, security forces accompanied the EDS team for the burial of 2 confirmed cases at the Bundji public cemetery, the PCI teams for the decontamination of CM Emmanuel Dalson (AS Tamende), CS La Grâce (AS Beningule), CS Kasanga and a household in AS Kasanga, as well as the EDS teams for the swab of 2 community deaths in Kavisa and Rugenda (ZS of Katwa); the security threat persists against the PoC providers on the Butembo axis.

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

- **In Ituri**, 12 traders (7 men, 2 women, 2 girls and 1 boy) were sensitized on the PSEAH in the Bunia health zone and the display of messages as well as the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa and Fataki; only 13 out of 36 health zones (36.1%) know the reporting mechanism.
- **In Haut-Uélé**, 190 students and teachers from the ITM of Isiro, 22 members of the Red Cross of Wamba, who signed the code of good conduct, 10 taxi drivers from Wamba and 7 motorcycle repairers from Boma Mangbetu were sensitized on the PSEA and the reporting circuit to the toll-free numbers (495555/151); the 6 ZS covered know the reporting mechanism (100%).

2.13.2. Challenges

At the national level, only 1 out of 7 DPS (14.3%) reported its PSEA activities; in Ituri, activities do not yet cover all affected health zones, mass awareness materials are lacking and the mapping of stakeholders remains incomplete.

MESSAGES CLES

ÿ In the provinces of Ituri, North Kivu and Haut-Uélé, it is observed active community transmission while sporadic notification is observed in Tshopo province.

2. In the last 24 hours.

ÿ Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (80.4% vs. 79.7%) and warrants further attention to prevent uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



TASK FORCE PRESIDENTIAL EBOLA 17
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